NCCP 00244 · VERSION 5

Your treatment, explained

Previously treated metastatic colorectal cancer · NCCP 00244 v5 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Regorafenib (Stivarga) monotherapy

28-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shivering, sudden illness, yellow eyes or skin, dark urine, severe or persistent abdominal pain, black or bloody stools, new bleeding, severe diarrhoea or inability to drink. Severe chest pain, major breathing difficulty, heavy bleeding, collapse, seizures, or a severe headache with confusion or visual change need emergency help: 112 / 999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- A 28-day tablet cycle
- Your 28-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- What this treatment is for
- Your tablet and rest-day plan
- Your tests and who follows them
- Skin, eating and daily activities
- Symptoms needing urgent help
- Other medicines and procedures
- Pregnancy prevention, storage and handling
- Your choices and practical support
- Supportive medicines

Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

Your targeted medicine and what it does

Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

ORAL MULTIKINASE INHIBITOR

Regorafenib

Stivarga



Blocks several signals involved in tumour growth and blood-vessel formation

Regorafenib blocks several protein switches called kinases. These help cancer cells grow and help blood vessels support a tumour. The medicine also affects normal tissues, so skin, liver, blood pressure and other symptoms need close review.

Growth and blood-vessel signals → Regorafenib blocks several kinases → Tumour-supporting signals are reduced

What to watch for

Report sore hands or feet early. Attend liver and blood-pressure checks even if you feel well, and get urgent help for serious new symptoms.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Head / energy

Regorafenib

Fatigue can affect daily life. Severe headache with confusion, visual change or a seizure needs emergency help.

3 Heart / blood pressure

Regorafenib

Blood pressure can rise without symptoms. Attend checks; chest pain, fainting or severe breathlessness needs emergency help.

5 Liver

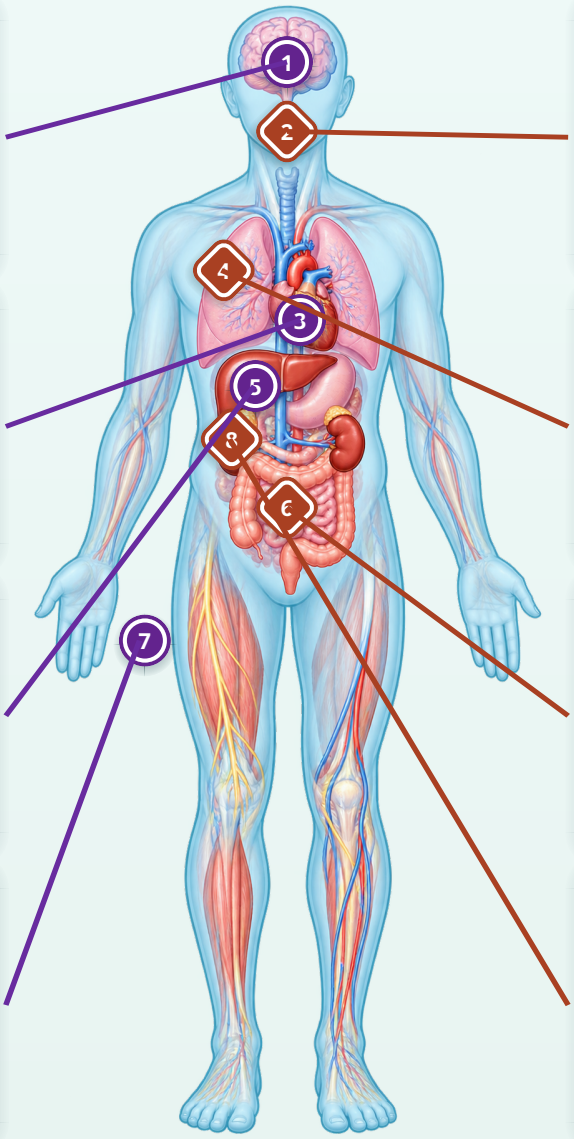
Regorafenib

Liver injury may first appear in blood tests. Yellow skin or eyes, dark urine or confusion needs immediate advice.

7 Hands / feet / skin

Regorafenib

Pain, redness, blisters or thickened skin can limit walking or hand use. Report early and follow your skin-care plan.



2 Mouth / appetite

Regorafenib

Soreness, ulcers, taste changes or poor appetite can affect eating. Report difficulty drinking or weight loss.

4 Blood / infection

Regorafenib

Bleeding, bruising or infection can occur. Fever or sudden illness needs urgent advice; heavy bleeding needs emergency help.

6 Bowel / abdomen

Regorafenib

Diarrhoea can cause dehydration. Severe or persistent pain, black stools or blood in stools needs urgent help.

8 Kidneys / fluids

Regorafenib

Vomiting or diarrhoea can reduce fluid intake and urine. Very little urine or inability to drink needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

A 28-day tablet cycle

28-DAY CYCLE

21 tablet days, then seven rest days

D1–21 D22–28

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

There is no fixed total number of cycles. Treatment continues while benefit and tolerability justify it. Your team reviews symptoms, blood tests, scans and your preferences; it may pause, reduce or stop the medicine.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Checks before, during and after treatment

Some tests are required for everyone; ECG/QT, pancreatic enzymes, INR and extra checks depend on risk, symptoms or other medicines. There is no single mandatory imaging or tumour-marker interval for all patients.

Check	Before starting	First six weeks	Up to two months	Later cycles	After / stopping
Liver blood tests	Baseline liver profile	Every two weeks	Every two weeks	Monthly or as clinically indicated	Follow any abnormal results until reviewed / resolved
Blood pressure	Baseline; control existing hypertension	Weekly	Before each cycle after week six; extra if needed	Before each cycle or as indicated	Follow the team's plan; do not stop BP tablets yourself
Blood count and kidneys	Baseline FBC and renal profile	Regular tests as arranged	Continue scheduled monitoring	Regular tests as arranged	Review residual blood-count, fluid or kidney problems
Thyroid and additional tests	Thyroid function; ECG/QT if at risk; pancreatic enzymes if indicated	Thyroid / pancreas tests if indicated; ECG / heart-rate review if needed	If on warfarin: close INR checks	Further tests according to symptoms and medicines	Follow abnormalities and symptoms individually

Check	Before starting	First six weeks	Up to two months	Later cycles	After / stopping
Symptoms and cancer response	Agree aim, baseline symptoms and review plan	Review hands/feet, mouth, bowel, appetite, energy and wounds	Review tolerability and any interruptions	Scans / markers according to the individual cancer plan	Agree follow-up, next treatment or supportive care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025 · EMA Stivarga · current product information and patient leaflet · checked 5 November 2026

THE WHOLE TREATMENT JOURNEY

Regorafenib (Stivarga) monotherapy: treatment and checks

Previously treated metastatic colorectal cancer

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	First six weeks	Up to two months	Later cycles	After / stopping
Liver blood tests NCCP 00244	Baseline liver profile	Every two weeks	Every two weeks	Monthly or as clinically indicated	Follow any abnormal results until reviewed / resolved
Blood pressure NCCP 00244	Baseline; control existing hypertension	Weekly	Before each cycle after week six; extra if needed	Before each cycle or as indicated	Follow the team's plan; do not stop BP tablets yourself
Blood count and kidneys NCCP 00244	Baseline FBC and renal profile	Regular tests as arranged	Continue scheduled monitoring	Regular tests as arranged	Review residual blood-count, fluid or kidney problems
Thyroid and additional tests NCCP 00244	Thyroid function; ECG/ QT if at risk; pancreatic enzymes if indicated	Thyroid / pancreas tests if indicated; ECG / heart-rate review if needed	If on warfarin: close INR checks	Further tests according to symptoms and medicines	Follow abnormalities and symptoms individually
Symptoms and cancer response NCCP 00244	Agree aim, baseline symptoms and review plan	Review hands/feet, mouth, bowel, appetite, energy and wounds	Review tolerability and any interruptions	Scans / markers according to the individual cancer plan	Agree follow-up, next treatment or supportive care

What each check is for

1

21 tablet days, then seven rest days

28-day cycle

Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00244

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

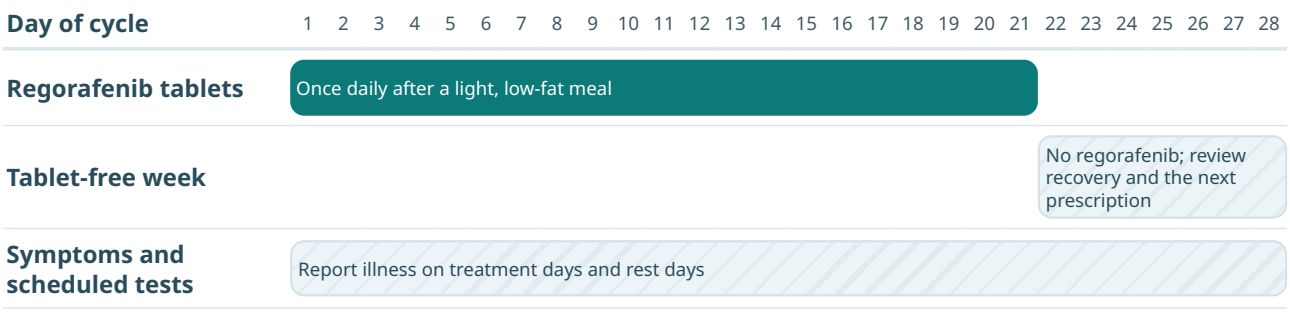
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.34

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.



Before the first tablet: Confirm consent, baseline tests, pharmacy teaching, the light-meal routine, skin care and working urgent contact numbers.

Longer-term review: There is no fixed final cycle. Review benefit and side effects with the team and agree what follows if treatment stops.

Early weeks have extra checks: The early liver and blood-pressure schedule continues across tablet-free days. A calendar stamp or a tablet supply does not establish that treatment is safe to restart.

> **Why these bands stay broad:** The bars show the prescribed tablet and rest days. They do not predict when side effects will happen or establish safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025 ↗

> **There are no guaranteed "safe days".** Use your oncology unit's urgent contact plan throughout the cycle, including tablet-free days. Symptoms can need attention before the next blood test or appointment.

Read the whole cycle as text

Day 1–7: Start with a clear calendar

Take the prescribed once-daily tablets after a light, low-fat meal. Early blood-pressure checks and a hand/foot care plan are important. Confirm the first blood-pressure and liver-test appointments before leaving.

Day 8–21: Continue tablets and early monitoring

Keep taking the prescribed dose unless the team changes it. Symptoms may build over time; hands, feet, appetite, bowel symptoms and energy need review. Report new problems early; do not wait for the end of the tablet block.

Day 22–28: Tablet-free week

Take no regorafenib. Side effects and abnormal tests can persist during the break. The next cycle may need a delay or a different dose. Complete arranged tests and confirm the next prescription and start date with the team.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Prescription and tablet calendar

Confirm the exact daily tablet number, Days 1–21 and Days 22–28, blood-test appointments and who authorises the next cycle. A clinician may prescribe a lower starting dose or change it after side effects; follow the issued prescription.



STEP 2

Taking tablets with food

Swallow the prescribed tablets whole with water at the same time each day after a light meal containing less than 30% fat. Ask the pharmacist for a practical meal example. Follow your own tablet label and calendar; do not change the dose yourself. Avoid grapefruit juice.



STEP 3

Missed or vomited doses

If you miss a dose, take it as soon as you remember on the same day. Do not take two doses in one day or make up the dose the next day. If you vomit after taking regorafenib, do not take extra tablets; tell the team. If unsure, contact the pharmacist or oncology team.



STEP 4

Storage and handling

Keep tablets in their original bottle, tightly closed, away from children and protected from moisture. Record when the bottle is first opened; the leaflet says to discard remaining tablets seven weeks after opening. Ask the pharmacist about safe disposal and help if you cannot swallow whole tablets.



STEP 5

Accidental extra tablets

Seek medical advice straight away if you take more than prescribed. Take the pack and current calendar with you.



STEP 6

Visits and practical support

This is tablet treatment without an infusion or take-home pump, but it still needs regular visits and tests. Ask who arranges appointments and results, and tell the team about travel, work or caring needs before changing the schedule.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Hands and feet

Inspect your hands and feet regularly and tell the team about pain, redness, swelling or blisters early. Ask for the unit's skin-care plan; it may include moisturising creams and protection or cushioning for hands and feet. Do not wait until walking or everyday hand use becomes difficult.

2 Sickness and bowel support

Regorafenib has minimal-to-low oral sickness risk; routine premedication is not usually required. Agree any anti-sickness and diarrhoea medicines with the team. Repeated vomiting, diarrhoea, poor intake or reduced urine needs prompt advice, even if you already have supportive medicines.

3 Mouth care, food and energy

Report mouth soreness, poor appetite, weight loss or fatigue affecting your usual activities. Ask about mouth care, nutritional help and a manageable activity/rest plan. Take the tablets after a light, low-fat meal and drink as advised for your health.

4 Blood pressure and liver checks

Attend tests even when you feel well. Liver tests are every two weeks for the first two months, then monthly or as clinically indicated. Blood pressure is checked weekly for six weeks, then before each cycle or as indicated. Confirm who reviews each result.

5 Medicines and procedures

The pharmacist needs a complete medicines and supplements list. Avoid grapefruit juice. Report planned surgery, dental procedures and wounds because treatment can affect healing. Tell the team if you take warfarin; closer INR checks are needed.

6 Pregnancy, fertility and feeding

Discuss fertility before treatment. Women who could become pregnant and men should use effective contraception during treatment and for at least eight weeks afterwards. Do not breastfeed during treatment. Contact the team promptly if pregnancy is possible.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

What this treatment is for

Confirm the diagnosis, treatment setting and individual goal.

Colorectal disease control

Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

Sources: nccp

Evidence and uncertainty

Median overall survival was 6.4 months with regorafenib and 5.0 months with placebo in the trial. These are group medians, not a prediction of your lifespan or a guaranteed extra 1.4 months. Side effects were more frequent with regorafenib. The team should relate the evidence to your health, previous treatment and priorities.

Sources: correct

How long it continues

There is no fixed total number of cycles. Treatment continues while benefit and tolerability justify it. Your team reviews symptoms, blood tests, scans and your preferences; it may pause, reduce or stop the medicine.

Sources: nccp

TREATMENT REFERENCE · 02

Your tablet and rest-day plan

Use the written prescription and calendar.

21 days on, seven days off

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

Sources: nccp

A light meal and the same daily time

Swallow the prescribed tablets whole with water at the same time each day after a light meal containing less than 30% fat. Ask the pharmacist for a practical meal example. Follow your own tablet label and calendar; do not change the dose yourself. Avoid grapefruit juice.

Sources: nccp · smpc

Missed or vomited doses

If you miss a dose, take it as soon as you remember on the same day. Do not take two doses in one day or make up the dose the next day. If you vomit after taking regorafenib, do not take extra tablets; tell the team. If unsure, contact the pharmacist or oncology team.

Sources: nccp · smpc

Extra tablets or swallowing difficulty

If you accidentally take too much, get medical advice straight away. Ask the pharmacist for help before altering or crushing a tablet.

Sources: smpc

TREATMENT REFERENCE · 03

Your tests and who follows them

Feeling well does not replace the early monitoring schedule.

Before treatment

Blood count, kidney/liver tests, thyroid function and blood pressure are checked. ECG/QT is used for patients at risk; pancreatic enzymes are measured when clinically indicated.

Sources: nccp

During treatment

Baseline blood count, kidney/liver profile, thyroid function and blood pressure. ECG/QT if at risk and pancreatic enzymes if indicated. During treatment: regular blood count and renal profile; liver profile every two weeks for two months, then monthly or as indicated; BP weekly for six weeks, then before each cycle or as indicated. Thyroid, pancreas and cardiac checks as indicated; close INR monitoring with warfarin. Scans and markers follow the Consultant's individual plan.

Sources: nccp

After a pause or after stopping

The team decides when to repeat abnormal tests and how to follow persistent side effects. Ask for the next review date and who remains your contact. Do not restart after a treatment hold without explicit instructions.

Sources: nccp · smpc

Scans and tumour markers

Your Consultant sets disease-assessment tests and timing for your situation. This regimen does not impose one fixed scan or tumour-marker timetable.

Sources: nccp

TREATMENT REFERENCE · 04

Skin, eating and daily activities

Early reporting can help prevent symptoms from becoming harder to manage.

Hands and feet

Inspect your hands and feet regularly and tell the team about pain, redness, swelling or blisters early. Ask for the unit's skin-care plan; it may include moisturising creams and protection or cushioning for hands and feet. Do not wait until walking or everyday hand use becomes difficult.

Sources: nccp · smpc

Sickness and bowel support

Regorafenib has minimal-to-low oral sickness risk; routine premedication is not usually required. Agree any anti-sickness and diarrhoea medicines with the team. Repeated vomiting, diarrhoea, poor intake or reduced urine needs prompt advice, even if you already have supportive medicines.

Sources: nccp · smpc

Mouth care, food and energy

Report mouth soreness, poor appetite, weight loss or fatigue affecting your usual activities. Ask about mouth care, nutritional help and a manageable activity/rest plan. Take the tablets after a light, low-fat meal and drink as advised for your health.

Sources: nccp · smpc

Thyroid, voice and other changes

Tell the team about worsening tiredness, feeling unusually cold, a hoarse voice, numbness or tingling, cramps or other persistent changes. The medicine can affect thyroid function, nerves and blood salts; symptoms may need tests or supportive treatment.

Sources: smpc

Driving and work

Do not drive or use machinery if symptoms affect concentration or reaction time. Discuss fatigue, painful hands/feet and your work or caring responsibilities with the team.

Sources: smpc

TREATMENT REFERENCE · 05

Symptoms needing urgent help

Frequency and urgency are different: a rare effect can still need immediate care.

Liver warning signs

Yellow skin or eyes, dark urine, or new confusion needs urgent medical advice. Liver injury can also start without obvious symptoms, which is why scheduled blood tests matter.

Sources: nccp · smpc

Bleeding, bowel problems and dehydration

Get urgent help for black or bloody stools, vomiting or coughing blood, severe or persistent abdominal pain, significant new bleeding, repeated vomiting or diarrhoea, or very little urine. Heavy bleeding or collapse needs 112 / 999.

Sources: smpc

Heart and neurological symptoms

Call 112 / 999 for chest pain with breathlessness or fainting, severe breathing difficulty, seizures, or a severe headache with confusion or visual changes. Do not wait for a routine blood-pressure appointment.

Sources: smpc

Infection

Fever, shivering or suddenly feeling unwell needs immediate oncology advice. New painful urination, a severe cough, shortness of breath, a sore throat or a red painful area can also signal infection.

Sources: smpc

Wounds, vessels and severe skin reactions

Tell the team about an unhealed wound or planned procedure. Seek urgent help for a widespread painful or blistering rash. New severe pain, swelling, neurological symptoms or sudden deterioration may need emergency assessment; the medicine can rarely affect blood-vessel walls or small blood vessels.

Sources: smpc

TREATMENT REFERENCE · 06

Other medicines and procedures

A complete medicines list helps the pharmacist check interactions.

Medicines and procedures

The pharmacist needs a complete medicines and supplements list. Avoid grapefruit juice. Report planned surgery, dental procedures and wounds because treatment can affect healing. Tell the team if you take warfarin; closer INR checks are needed.

Sources: nccp · smpc

Include non-prescription products

Tell the pharmacist about antibiotics, antifungals, epilepsy medicines, anticoagulants, pain medicines, supplements and herbal remedies, particularly St John's wort. Ask before starting or stopping anything. Avoid grapefruit juice.

Sources: smpc

Allergies and ingredients

Tell the team about medicine allergies, including reactions to similar medicines. Stivarga contains sodium and soya-derived lecithin; ask the pharmacist if these affect your care.

Sources: smpc

TREATMENT REFERENCE · 07

Pregnancy prevention, storage and handling

Agree personal precautions before the first dose.

Pregnancy, fertility and feeding

Discuss fertility before treatment. Women who could become pregnant and men should use effective contraception during treatment and for at least eight weeks afterwards. Do not breastfeed during treatment. Contact the team promptly if pregnancy is possible.

Sources: smpc

Storage and handling

Keep tablets in their original bottle, tightly closed, away from children and protected from moisture. Record when the bottle is first opened; the leaflet says to discard remaining tablets seven weeks after opening. Ask the pharmacist about safe disposal and help if you cannot swallow whole tablets.

Sources: smpc

TREATMENT REFERENCE · 08

Your choices and practical support

You can ask for clarification and reconsider treatment.

Consent and alternatives

Discuss the colorectal disease-control aim, likely benefit and uncertainty, alternatives including supportive care without regorafenib, early hand-foot skin reaction, blood-pressure and liver monitoring, fatigue, diarrhoea and appetite loss, serious bleeding/infection/heart/bowel risks, wound healing, interactions, pregnancy prevention and the option to reconsider. Agree who authorises changes and restarts.

Sources: nccp · smpc

Keep an urgent contact plan

Carry the treatment card and your current medicine list. Record the actual daytime and out-of-hours oncology numbers. Show the card when seeing another clinician and explain when you last took regorafenib.

Sources: nccp · smpc

A printable record

Use the passport to record prescribed changes, symptoms, tests and questions. It supports the conversation; it does not authorise tablets or replace the prescription.

Sources: nccp · smpc

Evidence for previously treated colorectal cancer

CORRECT compared regorafenib plus best supportive care with placebo plus best supportive care in 760 people with metastatic colorectal cancer after standard treatments.

Median overall survival was 6.4 months with regorafenib and 5.0 months with placebo in the trial. These are group medians, not a prediction of your lifespan or a guaranteed extra 1.4 months. Side effects were more frequent with regorafenib. The team should relate the evidence to your health, previous treatment and priorities.

Sources: correct · smpc

PREPARE FOR THE CONVERSATION

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber’s review. The patient page explains the checks and does not authorise treatment.

What if I miss a dose or vomit?

If you miss a dose, take it as soon as you remember on the same day. Do not take two doses in one day or make up the dose the next day. If you vomit after taking regorafenib, do not take extra tablets; tell the team. If unsure, contact the pharmacist or oncology team.

Does “targeted” mean there are no serious side effects?

No. The medicine blocks cancer-related signals but can also affect healthy tissues. Liver tests, blood-pressure checks and prompt symptom reporting are essential.

Can I use a diary stamp to decide to restart?

No. A stamp records a visit or completed cycle. Your team must confirm the dose and start date, especially after an interruption or abnormal test.

Why are checks so frequent at the beginning?

Blood pressure and liver tests can change before you notice symptoms. The early monitoring schedule helps the team identify problems and decide whether treatment needs to change.

Do I need radiotherapy or an infusion with these tablets?

This pathway is regorafenib monotherapy. An additional cancer treatment would require its own agreed plan and safety discussion.

Are the seven rest days safe days?

They allow a break from tablets, but problems can continue or develop then. Keep using the same urgent contact plan.

> **Try explaining the plan back:** Can you show the 21 tablet days and seven rest days, explain the meal and missed-dose instructions, identify the early liver and blood-pressure checks, and say whom you will call if unwell?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00244 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025

- EMA Stivarga · current product information and patient leaflet · checked 5 November 2026
- Grothey et al. CORRECT · colorectal regorafenib monotherapy · Lancet 2013

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6347/244_v5_Regorafenib_Monotherapy.pdf

https://www.ema.europa.eu/en/documents/product-information/stivarga-epar-product-information_en.pdf

https://pubmed.ncbi.nlm.nih.gov/23177514/